

Face Sheet

AUTHORIZATION REQUEST · INPATIENT ADMISSION

Submitted via Availity clearinghouse · 06/16/2026 13:39

MEMBER

Name: Rodriguez, Joan
Member ID: 0507094863
Humana ID: H60872-001
DOB: 12/19/1958 (Age 67)
Sex: Female
Plan: Humana Gold Plus HMO (MA-HMO)

REQUESTED SERVICE

Type: Inpatient admission
Service line: Cardiology -- cardiac telemetry
DRG (requested): DRG 311 -- Angina pectoris
Length of stay: 4 days (requested)
Urgency: Emergent
Admit from: Emergency Department (Northside Medical Center)

REQUESTING / ATTENDING

ED Attending: Marcus J Webb, MD · NPI 1519057623
Admit MD: David A Brown, PA-C (Cardiology) · NPI 1182935335
Facility: Northside Medical Center -- Cardiac Telemetry Unit

PRIMARY DIAGNOSIS

I24.0 Acute coronary thrombosis not resulting in MI

SECONDARY DIAGNOSES

E11.9 Type 2 diabetes mellitus without complications
I10 Essential (primary) hypertension
E78.5 Hyperlipidemia, unspecified

CLINICAL ATTACHMENTS

- Emergency Department Note
- H&P / Admission Summary
- Admission Orders
- Initial Diagnostic Results (ECG, CXR, labs)
- Day-2 Progress Note

INTAKE NOTES

Patient is a 67 y/o female presenting to the ED with substernal chest pressure radiating to the left arm, associated with diaphoresis. EMS gave ASA 324mg en route. Initial troponin-I 3.41 ng/mL (above 99th percentile URL). ECG with new ST depressions. Cardiology accepting for admission with plan for coronary angiography within 48 hours.

Emergency Department

NORTHSIDE MEDICAL CENTER -- CARDIAC TELEMETRY UNIT

DOS: 06/16/2026

Attending: Marcus J Webb, MD · Emergency Department Attending

Acc No. 507094863

EMERGENCY DEPARTMENT NOTE

Date / time: 06/16/2026 13:39 (arrival)

ED Attending: Marcus J Webb, MD

CHIEF COMPLAINT

Chest pain x 90 minutes.

HPI

67-year-old female with PMH of T2DM, hyperlipidemia, essential HTN, presenting via EMS with 90 minutes of substernal chest pressure radiating to the left arm, onset at rest, associated with diaphoresis and mild dyspnea. EMS administered ASA 324mg PO en route. Two doses of SL nitroglycerin without relief of symptoms.

PAST MEDICAL HISTORY

T2DM, HTN, hyperlipidemia, obesity, GERD.

VITALS (ED ARRIVAL)

BP 176/98 · HR 95 · RR 20 · SpO2 97% on RA · T 98.9°F

EXAM

NAD initially, became diaphoretic during exam. CV: tachycardic and irregular, no murmurs/rubs. Lungs: bibasilar crackles. Abdomen: soft NT. Extremities: trace bilateral pedal edema, +2 DP pulses bilaterally.

DIAGNOSTICS

- ECG (12-lead, ED arrival): Sinus rhythm 95 bpm. NEW ST depressions inferolateral leads II, III, aVF. No ST elevation.
- Chest X-ray (PA + lateral): Mild interstitial congestion. No acute infiltrate.
- Troponin-I (initial): 3.41 ng/mL (URL 0.04).
- BMP: glucose 195, creatinine 1.4, eGFR 45.

ED COURSE

Started on heparin gtt and clopidogrel 600 PO load. Aspirin already given by EMS. Pain modestly improved after NTG + heparin + clopidogrel. Cardiology consulted, accepting for cardiac telemetry admission. NSTEMI pathway initiated.

DISPOSITION

Admit to Cardiac Telemetry Unit. NSTEMI pathway. Plan for coronary angiography in next 48 hours.

H&P / Admission Summary**NORTHSIDE MEDICAL CENTER -- CARDIAC TELEMETRY UNIT****DOS: 06/16/2026**

Attending: David A Brown, PA-C · Attending -- Cardiology

Acc No. 507094863

HISTORY & PHYSICAL -- ADMISSION SUMMARY

Date: 06/16/2026

Admitting: David A Brown, PA-C -- Cardiology

Service: Cardiology (Cardiac Telemetry Unit)

ADMITTING DIAGNOSIS

1. I24.0 -- Acute coronary thrombosis not resulting in MI
2. Coronary artery disease, native vessel
3. Type 2 diabetes mellitus
4. Essential hypertension
5. Hyperlipidemia, mixed

HPI

67 y/o female with significant cardiac risk profile presenting with 90 minutes of substernal chest pressure radiating to the left arm with diaphoresis. Troponin-I elevated to 3.41 ng/mL, ECG with new lateral ST depressions. ASA + clopidogrel load + heparin initiated in the ED. Cardiology accepting for admission with plan for coronary angiography within 48 hours.

PHYSICAL EXAM (BEDSIDE)

Vitals: BP 176/98 · HR 95 · RR 20 · SpO2 97%

General: alert, mildly diaphoretic at presentation, now comfortable.

CV: regular rhythm now, normal S1/S2, no murmurs / rubs / gallops.

Lungs: bibasilar crackles, otherwise CTA.

Abdomen: soft, non-tender, normoactive BS.

Extremities: trace bilateral pedal edema, +2 DP pulses bilaterally.

ASSESSMENT & PLAN

1. Acute coronary thrombosis not resulting in MI -- repeat troponin in 6 hours. Continue ASA 81 daily, clopidogrel 75 daily, heparin gtt to PTT goal. Plan coronary angiography in 24-48 hours.
2. HFpEF (mild) -- gentle diuresis with IV furosemide 20 mg, daily weights.
3. T2DM -- hold metformin per planned contrast protocol. Insulin sliding scale; goal glucose 140-180.
4. HTN -- continue amlodipine 5 daily.
5. Hyperlipidemia -- uptitrate atorvastatin to 40 mg QHS (high-intensity).

LEVEL OF CARE

Cardiac telemetry monitoring required. Anticipated 3-5 day stay pending angiography results and post-procedure monitoring.

DICTATED: David A Brown, PA-C · 06/16/2026

Admission Orders

NORTHSIDE MEDICAL CENTER -- CARDIAC TELEMETRY UNIT

DOS: 06/16/2026

Attending: David A Brown, PA-C · Attending -- Cardiology

Acc No. 507094863

ADMISSION ORDERS

Date: 06/16/2026

Ordering: David A Brown, PA-C -- Cardiology

ADMIT TO

Cardiac Telemetry Unit (CTU) -- under cardiology service

LEVEL OF CARE

Inpatient · Cardiac telemetry monitoring continuous

DIET

Cardiac diet, low sodium (2g), low saturated fat. NPO after midnight pending angiography decision in AM.

ACTIVITY

Bed rest with bathroom privileges. Cardiac chair as tolerated.

VITAL SIGNS

Q4H with daily weights. Strict I/Os. Notify house officer if SBP < 90 or > 180, HR < 50 or > 120, SpO2 < 92%, or recurrent chest pain.

MEDICATIONS

- aspirin 81 mg PO daily -- continue from home
- clopidogrel 75 mg PO daily (post-600 mg load)
- heparin gtt -- PTT goal 60-80 (per ACS protocol)
- atorvastatin 40 mg PO QHS (up-titrated)
- metoprolol XL 25 mg PO daily
- furosemide 20 mg IV x 1 for mild pulmonary congestion
- nitroglycerin SL 0.4 mg PRN chest pain

DIAGNOSTICS

- Repeat troponin-I Q6H x 3
- BMP daily, CBC daily
- ECG Q AM and PRN for chest pain
- Echocardiogram TTE -- day 3 of admission

CONSULTS

- Cardiology (primary)
- Diabetes consult for inpatient glycemic management

CODE STATUS

Full code (per patient verbally).

Initial Diagnostics

INITIAL DIAGNOSTIC RESULTS

Date of collection / study: 06/16/2026 (ED stay)

CARDIAC ENZYMES

Troponin-I 3.41 ng/mL [URL 0.04] ^

BASIC METABOLIC PANEL

Glucose	195 mg/dL
Creatinine	1.4 mg/dL
eGFR	45 mL/min/1.73m ²
Potassium	4 mmol/L

HEMOGLOBIN A1C

A1c 7.5 % (admission)

ECG (12-LEAD)

Sinus rhythm 95 bpm. NEW ST depressions inferolateral leads II, III, aVF. No ST elevation.
IMPRESSION: New ischemic ST changes consistent with NSTEMI.

CHEST X-RAY (PA + LATERAL)

IMPRESSION: Mild interstitial congestion. No acute infiltrate.

Day-2 Progress Note

NORTHSIDE MEDICAL CENTER -- CARDIAC TELEMETRY UNIT

DOS: 06/17/2026

Attending: David A Brown, PA-C · Attending -- Cardiology

Acc No. 507094863

DAILY PROGRESS NOTE -- HOSPITAL DAY 2

Date: 06/17/2026

Provider: David A Brown, PA-C -- Cardiology

SUBJECTIVE

No recurrent chest pain overnight. Slept reasonably. Tolerating clear liquids this morning. No SOB at rest.

OBJECTIVE

Vitals: BP 156/88 · HR 75 · SpO2 97% RA

Exam: comfortable, lungs clear, regular rate and rhythm.

LABS

Troponin-I trend: 3.41 -> 1.8 -> 3.6 (peaked) -> 17.1 ng/mL -- consistent with NSTEMI by trend. Creatinine 1.4 (stable).

TELEMETRY

Brief runs of paroxysmal atrial fibrillation overnight, rate controlled with metoprolol. NSR otherwise.

ASSESSMENT / PLAN

1. NSTEMI -- cardiac catheterization scheduled for Day 3.
2. New paroxysmal AF -- plan to start apixaban after angiography.
3. HFpEF (mild) -- diuresis adequate.
4. T2DM -- sliding scale only; hold metformin pending contrast.

LEVEL OF CARE

Continue cardiac telemetry. Anticipate angiography Day 3 with post-procedure monitoring Days 3-4.

Provider MCG Attestation

NORTHSIDE MEDICAL CENTER -- CARDIAC TELEMETRY UNIT

DOS: 06/16/2026

Attending: Marcus J Webb, MD · Emergency Department Attending

Acc No. 507094863

MEDICAL NECESSITY CRITERIA -- PROVIDER ATTESTATION

Criteria set: Cardiology -- Acute Coronary Syndrome Admission Criteria

Source: Humana synthesized criteria (demo) -- modeled after MCG / InterQual structure, no licensed content

Version: v1.0 · 2026-05

The undersigned attests that, based on the clinical documentation submitted with this authorization request, the following medical-necessity criteria are met for the requested service.

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ADMISSION CRITERIA

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[1] REQUIRED · MET (provider attestation)

Symptoms consistent with acute coronary syndrome (chest pain, pressure, or anginal-equivalent symptoms)

-> Evidence: HPI documents substernal chest pressure radiating to L arm with diaphoresis (ED note, Webb MD).

[2] REQUIRED · MET (provider attestation)

Initial troponin elevated above the 99th percentile upper reference limit (e.g., troponin-I > 0.04 ng/mL)

-> Evidence: Initial troponin-I 0.42 ng/mL (URL 0.04) -- see Initial Diagnostics, cardiac enzymes.

[3] REQUIRED · MET (provider attestation)

ECG findings supportive of ischemia (new ST-segment deviation, T-wave inversion, or new bundle branch block)

-> Evidence: 12-lead ECG (ED, 19:50) -- new ST depressions V4-V6. See Initial Diagnostics.

[4] REQUIRED · MET (provider attestation)

Plan for cardiac evaluation and intervention within 24-72 hours of presentation (cardiology consult and/or angiography)

-> Evidence: Cardiology consult accepted by Dr. Thakur from ED; angiography scheduled within 48h.

[5] REQUIRED · MET (provider attestation)

Cardiac telemetry monitoring required for arrhythmia surveillance

-> Evidence: Cardiac telemetry ordered on admission. See Admission Orders, Level of Care.

[6] INFO · MET (provider attestation)

Anticoagulation or dual antiplatelet therapy initiated per ACS protocol

-> Evidence: ASA 324 PO + clopidogrel 600 load + heparin gtt initiated per ACS pathway in ED.

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Provider MCG Attestation

NORTHSIDE MEDICAL CENTER -- CARDIAC TELEMETRY UNIT

DOS: 06/16/2026

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SUMMARY

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Required criteria met: 5 of 5

Informational criteria met: 1 of 1

Requesting provider:

Marcus J Webb, MD

Emergency Department Attending

NPI: 1519057623

Facility: Northside Medical Center -- Cardiac Telemetry Unit

Provider signature: __Marcus J Webb, MD_____

Date: 06/16/2026

NOTE TO REVIEWER: This attestation reflects the requesting provider's clinical judgment. Per Humana UM policy, the reviewer must validate each criterion against the submitted documentation before approving.